

Medical AI Report

Priority Level: Medium

AI Summary:

Here's an analysis of the patient report:

* **Key symptoms:**

- * Mild fever
- * Fatigue
- * Headache (for 3 days)

* **Important findings:**

- * All basic lab results (Hemoglobin, WBC Count, Platelet Count, Blood Sugar) are within normal reference ranges.
- * Mild dehydration (as noted in the diagnosis).

* **Possible condition:**

- * Viral Fever with mild dehydration

* **Recommended next step:**

- * Follow-up with the doctor if symptoms persist after the recommended treatment (Paracetamol, fluids, rest).

Doctor Notes:

TEST MEDICAL REPORT

Sunrise Multi-Speciality Hospital

123 Health Avenue, Hyderabad, India

Phone: +91-9876543210

****Patient Name:**** Sarah Chen

****Age / Gender:**** 28 / Female

****Patient ID:**** MED-2026-1002

****Date of Visit:**** 15-05-2026

****Doctor:**** Dr. Rajesh Kumar

****Chief Complaint:**** Abdominal pain, nausea, vomiting (2 episodes), and watery diarrhea (4 episodes) for the past 24 hours. Patient reports consuming street food yesterday evening. Denies fever or blood in stool. Reports feeling weak and lightheaded.

****Patient Summary:****

28-year-old female presenting with acute onset of gastrointestinal symptoms including abdominal cramping, nausea, vomiting, and diarrhea for one day. Symptoms began approximately 12 hours after consuming street food. No significant past medical history. No known allergies. Currently feeling fatigued and experiencing mild orthostatic dizziness. Hydration status appears mildly compromised on examination (dry oral mucosa, reduced skin turgor). Abdomen is soft, tender to palpation in the epigastric and periumbilical regions, with hyperactive bowel sounds.

****Possible Diagnosis:****

- **Acute Gastroenteritis:**** Most likely viral or bacterial, given the sudden onset and association with food consumption.
- **Food Poisoning:**** Direct result of bacterial toxins or pathogens in contaminated food.

3. Dehydration secondary to fluid loss from vomiting and diarrhea.

****Suggested Tests:****

1. ****Complete Blood Count (CBC):**** To assess for signs of infection (elevated WBC) and hemoconcentration (due to dehydration).
2. ****Electrolyte Panel (Na, K, Cl, Bicarbonate):**** To evaluate for electrolyte imbalances due to significant fluid loss.
3. ****Stool Culture & Sensitivity:**** Recommended if symptoms are severe, persistent beyond 48-72 hours, or if there is suspicion of specific bacterial pathogens (e.g., travel history, immunocompromised state, bloody stools - though not present here, it's a general consideration).
4. ****Stool Ova and Parasites (O&P):**** If symptoms persist or travel history suggests parasitic exposure.

****Treatment Suggestions:****

1. ****Oral Rehydration Solution (ORS):**** Encourage frequent sips of ORS to prevent and treat dehydration.
2. ****Anti-emetic (e.g., Ondansetron 4mg ODT PRN):**** For symptomatic relief of nausea and vomiting, to facilitate oral fluid intake.
3. ****Probiotic Supplementation:**** May help restore gut flora and potentially shorten duration of diarrhea.
4. ****Dietary Modifications:**** Advise a bland diet (BRAT diet - Bananas, Rice, Applesauce, Toast) initially. Avoid dairy products, fatty, spicy, and high-fiber foods for 2-3 days.
5. ****Rest:**** Adequate rest is crucial for recovery.
6. ****Monitor for Worsening Symptoms:**** Advise patient to return immediately if symptoms worsen, develops high fever ($>102^{\circ}\text{F}$), severe abdominal pain, bloody stools, or signs of severe dehydration (e.g., decreased urination, severe dizziness, confusion).

7. ****Follow-up:**** Re-evaluation in 24-48 hours or sooner if symptoms do not improve or worsen.

****Basic Lab Results****

Test	Result	Reference Range	
:-----	:-----	:-----	
Hemoglobin	14.2 g/dL	12.0 - 16.0 g/dL	
WBC Count	12,500 / μ L	4,000 - 11,000 / μ L	
Platelet Count	2.8 Lakhs / μ L	1.5 - 4.5 Lakhs / μ L	
Blood Sugar	92 mg/dL	70 - 110 mg/dL	
Sodium	132 mEq/L	135 - 145 mEq/L	
Potassium	3.4 mEq/L	3.5 - 5.0 mEq/L	

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